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A woman is admitted at 36 weeks gestation following sudden episode of antepartum haemorrhage. The bleeding has ceased on admission. She is not pale. BP 120/80 and pulse is 76 beats/minute. The uterus is soft and not tender. There are no palpable uterine contractions. FHR is 140/minute. What is the next step of management?

- a) Ultrasound examination of the uterus
- b) Examine the cervix using Cusco's speculum
- c) Perform cardiotocograph
- d) Pelvic examination under general anesthesia
- e) Administer intravenous dexamethasone

This image shows one of the maneuvers used to correct breech presentation:



stocia:

- a) Internal rotation
- b) Zavanelli maneuver
- c) Delivery of posterior arm
- d) Screw maneuver

A pregnant woman is found to have excessive accumulation of amniotic fluid. Such polyhydramnios is likely to be associated with all of the following conditions EXCEPT:

- a) Twins pregnancy
- b) Oesophageal atresia
- c) Microcephaly
- d) Bilateral renal agenesis
- e) Open spina bifida

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A woman with dichorionic diamniotic twins attends antenatal clinic at 36 weeks. The first twin is in breech presentation and the second is in cephalic presentation. She has no other pregnancy complications. What is the most appropriate management?

- a) Perform internal podalic version
- b) Await spontaneous onset of labor
- c) Induce labor at 37 weeks
- d) Perform external cephalic version of the first
- e) Perform elective cesarean section at 37 weeks

All predispose to isoimmunization in Rh-negative female EXCEPT:

- a) Ectopic pregnancy
- b) Advanced maternal age
- c) Vaginal delivery
- d) Cesarean section
- e) Antepartum hemorrhage

A primipara whose period of amenorrhea is 39 weeks develops sudden severe abdominal pain, shock and tender uterus without vaginal bleeding. She is not in labor. What is the most likely diagnosis?

- a) Abruptio placenta
- b) Amniotic fluid embolism
- c) Chorioamnionitis
- d) Rupture uterus
- e) Red degeneration of fibroid

A woman complains of sudden onset of watery vaginal discharge at a period of amenorrhea 32 weeks. The fetus is in cephalic presentation with no uterine contractions. FHR 140 beats per minute. What is the first step in management?

- a) Commence broad spectrum antibiotics
- b) Commence intravenous corticosteroids
- c) Commence intravenous tocolysis
- d) Perform ultrasound scan to assess amniotic fluid volume
- e) Perform sterile speculum examination

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The pathognomonic sign of monochorionic diamniotic twins pregnancy by ultrasonography is:

- a) V sign
- b) Y sign
- c) U sign
- d) T sign
- e) Lambda sign

All of the following are components of biophysical profile EXCEPT:

- a) Amniotic fluid volume
- b) Oxytocin challenge test
- c) Fetal respiratory movement
- d) Non-stress test
- e) Fetal body movement

A 35 year old woman, G2 P1+0 presents to antenatal clinic at 35 weeks of pregnancy with sudden gush of watery vaginal discharge. Sample of pooled liquid turned red litmus paper blue and the ferning was present. The temperature of the patient was 38°C and her pulse was 102 per minute. What is the next step in management?

- a) Administer tocolytics
- b) Administer betamethasone
- c) Administer antibiotics
- d) Place cervical cerclage
- e) Administer magnesium sulphate

In which presentation the fetal head is deflexed?

- a) Compound presentation
- b) Face presentation
- c) Occipito-posterior position
- d) Occipito-anterior position
- e) Brow presentation

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True about frank breech:

- a) Buddha attitude
- b) Thigh extended, knee extended
- c) Thigh flexed, knee extended
- d) More common in multipara
- e) Both are flexed

A 25-year-old pregnant lady at 39 weeks of gestation. During labor, a hand is found beside the head in cephalic presentation. What is your diagnosis?

- a) Occipito-posterior position
- b) Face presentation
- c) Brow presentation
- d) Occipito-anterior position
- e) Compound presentation

The following techniques are used for delivery of the aftercoming head in breech presentation except:

- a) Forceps delivery
- b) Lovset's maneuver
- c) Mauriceau-Smellie-Veit technique
- d) Burns-Marshall's technique
- e) Jaw flexion shoulder traction

What is the denominator of face presentation?

- a) Orbital margin
- b) Chin
- c) Frontal bone
- d) Fetal mouth
- e) Fetal nose

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Cesarean section is mandatory in the following presentation:

- a) Vertex
- b) Brow
- c) Occipito-posterior
- d) Face
- e) Breech

Treatment of postpartum hemorrhage includes all EXCEPT:

- a) Oxytocin
- b) Syntometrine
- c) Oestrogen
- d) Carbetocin

Which is not included in the management of the third stage of labor?

- a) Massage of the uterus
- b) Controlled and sustained cord traction
- c) Direct oxytocin injection after delivery of the shoulders
- d) Uterotonic within one minute of delivery
- e) Immediate cutting and cord clamping

What is the measurement of sub-occipito-bregmatic diameter?

- a) 9 cm
- b) 9.5 cm
- c) 10 cm
- d) 10.5 cm
- e) 11 cm

Ritgen maneuver is done in:

- a) For delivery of the head in breech presentation
- b) For delivery of the head in brow presentation
- c) For delivery of the legs in breech presentation
- d) For delivery of the head in normal labor
- e) Shoulder dystocia

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All of the following are physiological changes during pregnancy EXCEPT:

- a) Increased tidal volume
- b) Decreased thyroxin-binding globulin
- c) Increased glomerular filtration rate
- d) Increased fibrinogen
- e) Increased stroke volume

A 22-year-old female underwent suction evacuation for molar pregnancy. Beta HCG levels are persistently high 3 months following evacuation.

What is the next step of management?

- a) Monitor till HCG is zero
- b) Insertion of intrauterine device (IUD)
- c) Methotrexate
- d) Hysterectomy
- e) Follow up at 1 year

A woman is admitted to the antenatal ward at 35 weeks. Her blood pressure is 179/110 and the urine contains albumin. She has generalized seizure during examination. An oropharyngeal airway is inserted. What is the most appropriate next step in management?

- a) Admit to the intensive care unit
- b) Give 10 mg of intravenous hydralazine
- c) Immediate transfer for cesarean section
- d) Insert an indwelling catheter
- e) Give 4 grams bolus of magnesium sulphate

A primigravida presents in emergency room with painless bleeding at 12 weeks of gestation. Cervix is closed and normal. What is the most probable diagnosis?

- a) Inevitable abortion
- b) Placenta previa
- c) Septic abortion
- d) Threatened abortion
- e) Incomplete abortion

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In case of diabetes mellitus with pregnancy:

- a) The best screening test is urine glucose
- b) Congenital malformations are unrelated to diabetic control
- c) Insulin requirement decreases as pregnancy advances
- d) There is increased incidence of hydramnios and preeclampsia
- e) Oral hypoglycemics are preferred to insulin therapy

The time at which cardiac output is highest during pregnancy is:

- a) In the first trimester
- b) In the second trimester
- c) In the third trimester
- d) During labor
- e) Immediately postpartum

All of the following are structures of the fetal membranes EXCEPT:

- a) Decidua
- b) Allantois
- c) Yolk sac
- d) Connecting stalk
- e) Chorion

A primigravida woman with mitral stenosis without pulmonary hypertension is in labor at 39 weeks. She has dyspnea only on severe exertion. Fetus is in cephalic presentation, cervix is 5 cm dilated, and she has 2 uterine contractions in 10 minutes. Which of the following should be avoided in this patient?

- a) Use of epidural analgesia for pain relief
- b) Ventouse delivery if second stage prolonged
- c) Placing the patient in semi-sitting position
- d) Use of ergometrine in third stage of labor
- e) Active management of third stage of labor

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A woman presents with abdominal pain and nausea with amenorrhea for 5 weeks. Ectopic pregnancy is diagnosed if:

- a) Beta HCG > 1000 IU/L with endometrial thickness of 14 mm
- b) Beta HCG > 2000 IU/L with no gestational sac in uterus by transvaginal US
- c) Beta HCG < 1000 IU/L with empty uterus on transvaginal US
- d) Serum progesterone > 25 ng/dL
- e) Serum prolactin > 20 ng/dL

A patient has microcytic anemia with hemoglobin 9 and normal iron stores. What is the most likely diagnosis?

- a) Thalassemia
- b) Iron deficiency anemia
- c) Vitamin B6 deficiency
- d) Vitamin B12 deficiency
- e) Folate deficiency

All of the following are true regarding cephalohematoma EXCEPT:

- a) Develops hours after birth
- b) Well-defined edges
- c) Normal overlying skin
- d) Disappears within two days
- e) Localized over one bone

G2 para 1+0 who had uncomplicated lower segment cesarean section in her first pregnancy is in labor. She is allowed a trial of vaginal delivery. What is the earliest warning sign of scar rupture?

- a) Fetal distress
- b) Fresh vaginal bleeding
- c) Severe continuous abdominal pain
- d) Maternal tachycardia

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The uterus becomes a pelvic organ after delivery by:

- a) Four weeks
- b) Two weeks
- c) Three weeks
- d) One week

Ventouse in second stage of labor is contraindicated in:

- a) Persistent occipito-posterior position
- b) Uterine inertia
- c) Heart disease
- d) Preterm labor

Lochia in correct order during puerperium is:

- a) Alba, mucosa, serosa
- b) Alba, serosa, rubra
- c) Rubra, serosa, alba
- d) Serosa, rubra, alba

All of the following are true for episiotomy EXCEPT:

- a) Involvement of the anal sphincter is classified as 4th degree perineal tear
- b) Midline episiotomy bleeds less, easier to repair, and heals more quickly
- c) Can be either midline or mediolateral
- d) Allows widening of the birth canal

The nerve roots affected in Klumpke's palsy are:

- a) C7, C8, T1
- b) C8, T1
- c) C1, C2, T1
- d) C7, C8, T2
- e) C5, C6

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Forceps delivery can be done in all the following EXCEPT:

- a) Maternal heart disease
- b) Aftercoming head in breech delivery
- c) Deep transverse arrest
- d) Persistent mento-posterior

Regarding indications of cesarean hysterectomy, which of the following statements is true:

- a) Placenta accreta encountered during cesarean section
- b) Multiple uterine fibroids in a relatively old pregnant woman who had completed her family
- c) The mere presence of Couvelaire uterus
- d) Uncontrolled postpartum hemorrhage
- e) Operable carcinoma of the cervix during pregnancy

Prerequisites of outlet forceps include all the following EXCEPT:

- a) Uterus contracting
- b) Engaged head
- c) Station 0 or +1
- d) Fully dilated cervix

Your patient has microcytic anemia with a hemoglobin of 9 and normal iron stores. What is the most likely diagnosis?

- a) Folate deficiency
- b) Vitamin B12 deficiency
- c) Thalassemia
- d) Iron deficiency anemia
- e) Vitamin B6 deficiency

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All of the following are characters of HELLP syndrome EXCEPT:

- a) Thrombocytopenia
- b) Increased SGOT and SGPT
- c) Hemolytic anemia
- d) Hyperuricemia

Which of the following Leopold's maneuvers is shown in the image?

- a) First pelvic grip
- b) Fundal level
- c) Fundal grip
- d) Second pelvic grip
- e) Umbilical grip



All of the physiological changes during pregnancy EXCEPT:

- a) Decreased fibrinogen
- b) Decreased thyroxin binding globulin
- c) Increased stroke volume
- d) Increased tidal volume
- e) Increased glomerular filtration rate

Diagnosis of this physiological skin change in attached picture:

- a) Linea nigra pigmentation
- b) Palmer erythema
- c) Stria gravidarum
- d) Chloasma gravidarum



A primipara whose period of amenorrhea is 39 weeks develops sudden severe abdominal pain, shock, and tender uterus without vaginal bleeding. She is not in labor. What is the most likely diagnosis?

- a) Abruptio placenta
- b) Rupture uterus
- c) Amniotic fluid embolism
- d) Chorioamnionitis
- e) Red degeneration of fibroid

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All of the following are sure signs of pregnancy EXCEPT:

- a) Palpation of fetal movement
- b) Quickening
- c) Palpation of fetal parts
- d) Auscultation of fetal heart sound
- e) Umbilical soufflé

A 35-year-old woman, G2 P1+0, presents to antenatal clinic at 35 weeks of pregnancy with sudden gush of watery vaginal discharge. Sample of pooled liquid turned red litmus paper blue and ferning was present. The temperature of the patient was 38°C and her pulse was 102 bpm. What is the next step in management?

- a) Administer tocolytics
- b) Place cervical cerclage
- c) Administer betamethasone
- d) Administer magnesium sulphate
- e) Administer antibiotics

A primigravida woman with mitral stenosis with no pulmonary hypertension is in labor at 39 weeks. She has dyspnea only in severe exertion. Fetus is in cephalic presentation. Cervix dilatation is 5 cm. She has two uterine contractions in 10 minutes. Which of the following should be avoided in this patient:

- a) Active management of third stage of labor
- b) Use of ergometrine in the third stage of labor
- c) Ventouse delivery in the second stage of labor if delivery does not occur in half an hour
- d) Use of epidural analgesia for pain relief

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A 31-year-old G2P0 at 5 weeks' gestation presents for her first prenatal visit complaining of vague abdominal cramping and bleeding that is slightly less than her normal menstrual period. The most appropriate steps to evaluate this patient are:

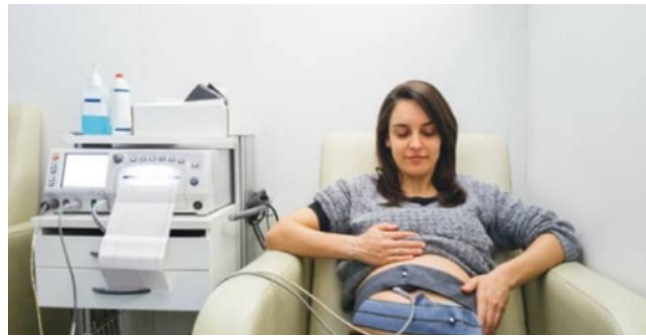
- a) Ultrasound (U/S) and CBC
- b) B-hCG and progesterone
- c) Quantitative B-hCG, then ultrasound
- d) Qualitative B-hCG and CBC
- e) Serial quantitative B-hCG

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What is the measurement of sub-occipito-bregmatic diameter?

- a) 10 cm
- b) 10.5 cm
- c) 9 cm
- d) 9.5 cm
- e) 11 cm

With reference to fetal heart rate, a non-stress test is considered reactive when:



- a) Two fetal heart rate accelerations are noted in 10 minutes
- b) One fetal heart rate acceleration is noted in 20 minutes
- c) Fetal heart accelerations during uterine contractions
- d) Three heart rate accelerations are noted in 30 minutes
- e) Two fetal heart rate accelerations are noted in 20 minutes

A female bleeds profusely soon after vaginal delivery. Placenta and membranes were expelled. IV fluids were initiated. Uterus is relaxed and fails to contract adequately after treatment with ecobolics. Her HR = 120/min and BP = 90/60. What is the next appropriate step of management?

- a) Ligate internal iliac artery
- b) Carry out balloon tamponade
- c) Ligate uterine arteries
- d) Do hysterectomy



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All of the following are true about pathological retraction ring EXCEPT:

- a) Occurs between upper and lower uterine segment
- b) Can be felt
- c) May be associated with intrauterine fetal death
- d) Can be relieved by anesthesia
- e) Associated with maternal dehydration

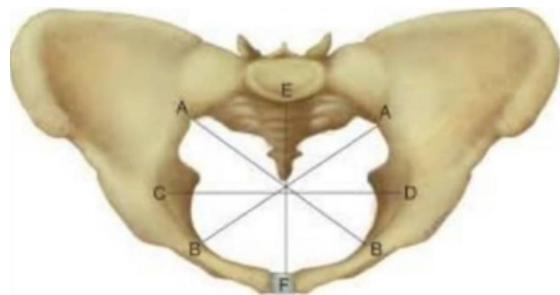
If the head is allowed to extend before this sign appears, the head will be delivered by:

- a) Suboccipito-frontal diameter
- b) Suboccipito-bregmatic diameter
- c) Submento-bregmatic diameter
- d) Mento-vertical diameter
- e) Occipito-frontal diameter



The smallest diameter of the true pelvis is:

- a) True conjugate diameter
- b) Diagonal conjugate diameter
- c) Intertuberous diameter
- d) Interspinous diameter



A primigravida complains of lower abdominal pain at a period of amenorrhea of 32 weeks. The fetus is in cephalic presentation. She has 1-2 uterine contractions per 10 minutes. Cervix is 2 cm dilated and 75% effaced with intact membranes. What is the preferred next step of management?

- a) Start intravenous broad-spectrum antibiotics
- b) Allow labor to progress normally
- c) Commence intravenous ecobolics
- d) Commence oral nifedipine
- e) Start intramuscular corticosteroids and intravenous ritodrine

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Mrs. Heba, Gravida 2 para 1+0, presented to hospital in labor pains. On examination she had 3 uterine contractions of 20 seconds in 10 minutes, cervical dilatation 10 cm with bearing down and FHR 145 bpm. What is the stage of labor?

- a) Stage I
- b) Stage II
- c) Stage III
- d) Stage IV

All of the following maneuvers are performed in shoulder dystocia EXCEPT:



- a) McRoberts position
- b) Fundal pressure
- c) Delivery of posterior arm
- d) Episiotomy
- e) Rotation of anterior shoulder

Which is not included in the management of the third stage of labor?

- a) Direct oxytocin injection after delivery of the shoulders
- b) Immediate cutting and cord clamping
- c) Massage of the uterus
- d) Controlled and sustained cord traction
- e) Uterotonic within one minute of delivery

Quizzes 40 G1

All of the following are maternal changes during pregnancy EXCEPT:

- a) Increased glomerular filtration rate
- b) Increased fibrinogen
- c) Increased cardiac output
- d) Increased alveolar residual volume

Regarding the process of fertilization, which of the following statements is correct?

- a) Fertilization occurs most commonly in isthmus of the fallopian tube
- b) Implantation occurs most commonly on posterior wall near the fundus
- c) Implantation begins 14 days after fertilization
- d) Morula enters uterus on day 7

The most common rheumatic heart disease associated with pregnancy is:

- a) Aortic stenosis
- b) Mitral stenosis
- c) Aortic regurgitation
- d) Mitral regurgitation

A primigravida 32 years old female, admitted to the antenatal ward at 35 weeks, her BP was 160/110. Her urine does not contain albumin. Which of the following drugs should NOT be used in her management?

- a) Labetalol
- b) Methyldopa
- c) Furosemide
- d) Hydralazine

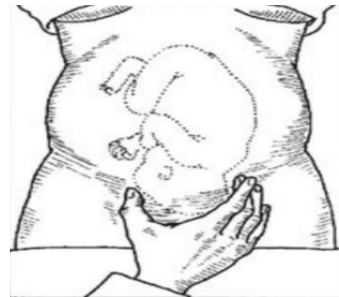
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A woman complains of right sided lower abdominal pain and mild vaginal bleeding after a period of amenorrhea of 6 weeks. Her general condition is satisfactory. Serum hCG is 2200 IU/ml. Transvaginal ultrasound reveals adnexal mass 20 mm with no cardiac activity, and no intrauterine pregnancy. The most appropriate management is:

- a) Treat with intramuscular methyl ergometrine
- b) Perform laparoscopic salpingostomy
- c) Treat with intramuscular methotrexate
- d) Perform laparoscopic salpingectomy

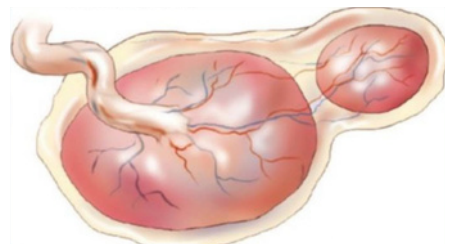
Which of the following Leopold's maneuvers is shown in the image:

- a) Second pelvic grip
- b) First pelvic grip
- c) Fundal grip
- d) Umbilical grip



Identify the placental abnormality shown in the figure:

- a) Succenturiate placenta
- b) Bilobed placenta
- c) Placenta membranacea
- d) Velamentous insertion of the cord



A 25 years old female, Gravida 5, para 0+3, with history of recurrent early pregnancy loss. Which of the following investigations is NOT ordered?

- a) Thyroid function test
- b) Genetic testing and karyotyping
- c) Testing for TORCH
- d) Testing for antiphospholipid syndrome

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A 42 years old female, para 4+2, has been diagnosed with complete vesicular mole. The treatment of choice would be:

- a) Chemotherapy
- b) Hysterectomy
- c) Chemo-radiotherapy
- d) Radiotherapy

The following are related to uterine soufflé EXCEPT:

- a) It can be heard in a big fibroid
- b) It is a soft blowing systolic murmur heard on the sides of the pregnant uterus
- c) The sound is synchronous with maternal pulse
- d) It is due to increased blood flow through the umbilical artery

The following vaccines are safe during pregnancy EXCEPT:

- a) Hepatitis B
- b) Influenza
- c) Rubella
- d) Tetanus
- e) Pneumococcus

With reference to fetal heart rate, a non-stress test is considered reactive when:

- a) Two fetal heart rate accelerations are noted in 20 minutes
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- a) Commence intramuscular corticosteroids and oral nifedipine
- b) Start intravenous broad spectrum antibiotics
- c) Start intramuscular corticosteroids and intravenous ritodrine
- d) Commence intravenous ecobolics
- e) Allow labor to progress normally

All of the following are ultrasound features of intrauterine fetal demise EXCEPT:

- a) Spalding sign
- b) Collapse of the thorax
- c) Gases in the intestine
- d) Turtle sign

Monozygotic twins have the following characteristics EXCEPT:

- a) More susceptible to twin to twin transfusion syndrome
- b) Has different blood groups
- c) Accounts for 20% of twins pregnancy
- d) Has the same gender

All of the following maneuvers are performed in shoulder dystocia EXCEPT:

- a) Delivery of posterior arm
- b) Fundal pressure
- c) Mc-Robert's position
- d) Episiotomy
- e) Rotation of anterior shoulder

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All of the following are sequelae of macrosomic baby EXCEPT:

- a) Meconium aspiration syndrome
- b) Erb's palsy
- c) Obstructed labor
- d) Shoulder dystocia
- e) Disseminated intravascular coagulopathy

The following abnormal findings can be found in IUGR baby EXCEPT:

- a) Oligohydramnios
- b) Change in Head/Abdomen ratio
- c) Absent uterine artery diastolic notch
- d) Absent umbilical artery diastolic blood flow

Which of the following diseases can cause polyhydramnios?

- a) IUGR
- b) Duodenal atresia
- c) Post maturity
- d) Placental insufficiency
- e) Absent kidneys

A woman is admitted to hospital with history of dribbling for 24 hours at a period of amenorrhea of 32 weeks. Speculum examination confirms dribbling. The liquor is offensive. Her temperature is 38°C. High C-reactive protein. No uterine contractions. FHR is 170 per minute. What is the most appropriate treatment?

- a) Administer broad spectrum antibiotics and corticosteroids and manage conservatively if the temperature decreases
- b) Perform cesarean section immediately after administering 1 dose of corticosteroids and broad spectrum antibiotics
- c) Augment labor with intravenous infusion of oxytocin

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Prerequisites of outlet forceps include all the following EXCEPT:

- a) Station 0 or +1
- b) Engaged head
- c) Fully dilated cervix
- d) Uterus contracting

G2 para 1+0 who had uncomplicated lower segment cesarean section in her first pregnancy in labor. She is allowed a trial of vaginal delivery. What is the earliest warning sign of scar rupture?

- a) Severe continuous abdominal pain
- b) Fresh vaginal bleeding
- c) Fetal distress
- d) Maternal tachycardia

The nerve roots affected in Klumpke's palsy are:

- a) C7, C8, T1
- b) C7, C8, T2
- c) C8, T1
- d) C5, C6
- e) C1, C2, T1

All of the following are true regarding cephalohematoma EXCEPT:

- a) Localized over one bone
- b) Disappears within two days
- c) Develops hours after birth
- d) Normal overlying skin
- e) Well defined edges

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The uterus becomes a pelvic organ after delivery by:

- a) One week
- b) Two weeks
- c) Four weeks
- d) Three weeks

Forceps delivery can be done in all EXCEPT:

- a) Maternal heart disease
- b) Aftercoming head in breech delivery
- c) Persistent mento-posterior
- d) Deep transverse arrest

All of the following are true for episiotomy EXCEPT:

- a) Midline episiotomy bleeds less, easier to repair, and heals more quickly
- b) Can be either midline or mediolateral
- c) Allows widening of the birth canal
- d) Involvement of the anal sphincter is classified as 4th degree perineal tear

Lochia in correct order during puerperium is:

- a) Serosa, rubra, alba
- b) Alba, serosa, rubra
- c) Rubra, serosa, alba
- d) Alba, mucosa, serosa

Regarding indications of cesarean hysterectomy, which of the following statements is NOT true:

- a) Uncontrolled postpartum hemorrhage
- b) Presence of Couvelaire uterus
- c) Multiple uterine fibroids in relatively old pregnant woman who had completed her family
- d) Placenta accreta encountered during cesarean section

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Ventouse in second stage of labor is contraindicated in:

- a) Uterine inertia
- b) Persistent occipito-posterior position
- c) Preterm labor
- d) Heart disease

Quizzes 40 G2

A woman complains of right-sided abdominal pain and mild vaginal bleeding for one day after a period of amenorrhea of 6 weeks. Her general condition is satisfactory. There is tenderness in the right iliac fossa. The urine pregnancy test is positive. Transvaginal ultrasound scan does not reveal any adnexal mass or intrauterine pregnancy. What is the next step of management?

- a) Perform two serum beta-HCG tests 48 hours apart
- b) Start treatment with methotrexate
- c) Repeat ultrasound scan after 7 days
- d) Perform laparoscopic examination

A woman who is nine weeks pregnant comes to the early pregnancy assessment unit complaining of severe nausea and occasional vomiting. She is not keen on drug therapy. What is your advice?

- a) Drink herbal tea
- b) Drink decaffeinated coffee
- c) Take a long break from work
- d) Drink ginger syrup
- e) Take up yoga

With regards to acute pyelonephritis in pregnancy, all are true EXCEPT:

- a) The incidence of left kidney involvement is more than the right kidney
- b) Most common isolate is E. coli
- c) Responds to cephalosporins
- d) More common in the second trimester of pregnancy

A 30-year-old P2+2 female has been diagnosed with complete vesicular mole. The treatment of choice would be:

- a) Total hysterectomy
- b) Expectant management
- c) Radiotherapy
- d) Suction evacuation

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A patient at 36 weeks gestation presents with hypertension, blurring of vision, and headache. Her blood pressure is persistently 180/120 with +3 proteinuria. How will you manage this patient?

- a) Start antihypertensive and follow up in outpatient department
- b) Admit the patient, start antihypertensive, and continue pregnancy till term
- c) Admit the patient and observe
- d) Admit the patient, start antihypertensive, MgSO_4 , and terminate the pregnancy

Primigravida at full term complains of fainting when lying down but feels well when turning to one side. This is due to:

- a) Increased abdominal pressure
- b) Increased intracranial pressure
- c) After heavy lunch
- d) Inferior vena cava compression

A 28-year-old female with 8 weeks amenorrhea complains of vaginal bleeding and lower abdominal pain. On ultrasound, there is a gestational sac with absent fetal parts. The diagnosis is:

- a) Blighted ovum
- b) Threatened abortion
- c) Ectopic pregnancy
- d) Corpus luteum cyst

What dose of glucose has been recommended by WHO for oral glucose tolerance test during pregnancy?

- a) 200 gm
- b) 50 gm
- c) 150 gm
- d) 100 gm
- e) 75 gm

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All the following are sure signs of pregnancy EXCEPT:

- a) Palpation of fetal parts
- b) Umbilical souffle
- c) Auscultation of fetal heart sounds
- d) Uterine souffle

In a pregnant woman with heart disease, all the following are to be done EXCEPT:

- a) IV methergin after delivery
- b) Shortening second stage of labor
- c) IV furosemide postpartum
- d) Prophylactic antibiotic

Which of the following diseases can cause polyhydramnios:

- a) Absent kidneys
- b) Post maturity
- c) IUGR
- d) Duodenal atresia
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- b) Oligohydramnios
- c) Absent umbilical artery diastolic blood flow

With reference to fetal heart rate, a non-stress test is considered reactive when:

- a) Two fetal heart rate accelerations are noted in 10 minutes
- b) One fetal heart rate acceleration is noted in 20 minutes
- c) Three heart rate accelerations are noted in 30 minutes
- d) Two fetal heart rate accelerations are noted in 20 minutes
- e) Fetal heart accelerations during uterine contractions

SUPPORT 41

Monozygotic twins have the following characteristics EXCEPT:

- a) Has the same gender
- b) More susceptible to twin-to-twin transfusion syndrome
- c) Accounts for 20% of twins pregnancy
- d) Has different blood groups

All of the following maneuvers are performed in shoulder dystocia EXCEPT:

- a) Fundal pressure
- b) McRobert's position
- c) Rotation of anterior shoulder
- d) Episiotomy
- e) Delivery of posterior arm

A primigravida complains of lower abdominal pain at 32 weeks of amenorrhea. The fetus is in cephalic presentation. She has 1-2 uterine contractions per 10 minutes. Cervix is 2 cm dilated and 75% effaced with intact membranes. What is the preferred next step of management?

- a) Start intravenous broad-spectrum antibiotics
- b) Start intramuscular corticosteroids and intravenous ritodrine
- c) Allow labor to progress normally
- d) Commence intramuscular corticosteroids and oral nifedipine

All of the following are sequelae of macrosomic baby EXCEPT:

- a) Disseminated intravascular coagulopathy
- b) Erb's palsy
- c) Shoulder dystocia
- d) Obstructed labor
- e) Meconium aspiration syndrome

SUPPORT 41

A woman is admitted to hospital with history of dribbling for 24 hours at 32 weeks of amenorrhea. Speculum exam confirms dribbling. Liquor is offensive. Temperature is 38°C. High CRP. No uterine contractions. FHR is 170 bpm. What is the most appropriate treatment?

- a) Administer broad-spectrum antibiotics and corticosteroids and manage conservatively if the temperature decreases
- b) Augment labor with intravenous infusion of oxytocin
- c) Perform cesarean section immediately after administering 1 dose of corticosteroids and broad-spectrum antibiotics

The following vaccines are safe during pregnancy EXCEPT:

- a) Hepatitis B
- b) Pneumococcus
- c) Rubella
- d) Influenza
- e) Tetanus

All of the following are ultrasound features of intrauterine fetal demise EXCEPT:

- a) Absent fetal movement
- b) Gases in the intestine
- c) Turtle sign
- d) Collapse of the thorax
- e) Spalding sign

A 30-year-old female G2 para 1+0 at 38 weeks gestation presents with labor pains. On examination, cervix is 5 cm dilated and the fetus was found to be in face presentation with left mento-anterior position. The next step of management is:

- a) Jaw flexion–shoulder traction
- b) Ventouse delivery
- c) Forceps
- d) Expectant management
- e) Cesarean section

SUPPORT 41

All of the following are true about pathological retraction ring EXCEPT:

- a) Occurs between upper and lower uterine segment
- b) May be associated with intrauterine fetal death
- c) Associated with maternal dehydration
- d) Can be felt
- e) Can be relieved by anesthesia

True about frank breech:

- a) Both are flexed
- b) Thigh extended, knee extended
- c) Thigh flexed, knee extended
- d) More common in multipara
- e) Buddha attitude

A 24-year-old G1 at term is taken to the operating room for a Cesarean section due to fetal distress. The newborn has an HR of less than 100, a slow irregular respiratory rate, flaccid muscle tone, a weak cry, and blue coloring of the body and limbs. What is the appropriate Apgar score?

- a) 2
- b) 5
- c) 4
- d) 3

Mrs. H, Gravida 2 para 1+0, presented to hospital in labor pains. On examination she had 3 uterine contractions of 20 seconds in 10 minutes, cervical dilatation 10 cm with bearing down and FHR 145 bpm. What is the stage of labor?

- a) Stage I
- b) Stage III
- c) Stage V
- d) Stage IV
- e) Stage II

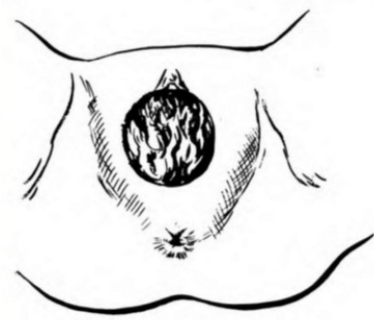
SUPPORT 41

The following techniques are used for delivery of the aftercoming head in breech presentation EXCEPT:

- a) Mauriceau Smellie Veit technique
- b) Burns Marshall technique
- c) Forceps delivery
- d) Lovset maneuver
- e) Jaw flexion shoulder traction

If the head is allowed to extend before this sign appears, the head will be delivered by:

- a) Submentobregmatic diameter
- b) Suboccipito-bregmatic diameter
- c) Suboccipito-frontal diameter
- d) Occipito-frontal diameter
- e) Mento-vertical diameter



The smallest diameter of true pelvis is:

- a) Intertuberous diameter
- b) Diagonal conjugate diameter
- c) True conjugate diameter
- d) Interspinous diameter

A female bleeds profusely soon after vaginal delivery. Placenta and membranes were expelled. IV fluids were initiated. Uterus is relaxed and fails to contract adequately after treatment with ecobolics. Her HR = 120 bpm, BP = 90/60. What is the next appropriate step of management?

- a) Ligate internal iliac artery
- b) Carry out balloon tamponade
- c) Ligate uterine arteries
- d) Apply B-Lynch suture
- e) Do hysterectomy

SUPPORT 41

A 32-year-old G2P1 presents to triage at 38 weeks and 5 days with ruptured membranes and meconium. She is having no contractions. The patient's first baby was delivered by Cesarean at term for footling breech presentation. She had discussed the possibility of a trial of labor after Cesarean section (TOLAC) during her prenatal visits and desires to proceed with this. What is this patient's greatest risk if she undergoes induction?

- a) Placental abruption
- b) Uterine rupture
- c) Pulmonary embolism
- d) Cord prolapse
- e) Meconium aspiration

A 31-year-old patient gravida 3 para 1+1, previous one spontaneous vaginal delivery, pregnant 41 weeks, came to the delivery room for assessment for induction of labor. On vaginal examination, cervix was anterior, soft in consistency, closed, fully effaced (cervical length = 0) with fetal head station at +1. What is her Bishop score?

- a) 5
- b) 10
- c) 4
- d) 8
- e) 6

All of the following are true regarding cephalohematoma EXCEPT:

- a) Normal overlying skin
- b) Localized over one bone
- c) Well defined edges
- d) Develops hours after birth
- e) Disappears within two days

SUPPORT 41

Advantages of ventouse over forceps include all of the following EXCEPT:

- a) Preferred for cardiac patients
- b) Less genital tract injuries
- c) More compression force
- d) Can be used before full cervical dilatation
- e) Can be used for non-engaged head

The following are indications of ergometrine EXCEPT:

- a) Subinvoluted uterus
- b) Inevitable abortion
- c) Uterine atony
- d) Cervical ripening
- e) Incomplete abortion

The nerve roots affected in Klumpke's palsy are:

- a) C7, C8, T1
- b) C1, C2, T1
- c) C8, T1
- d) C7, C8, T2
- e) C5, C6

The following are true about this instrument EXCEPT:



- a) Confirm if the uterus is anteverted or retroverted
- b) Obtain sample for endometrial biopsy
- c) Assess the length of uterine cavity
- d) Used preliminary to uterine curettage
- e) May cause uterine perforation if not used gently

SUPPORT 41

G2 para 1+0 who had uncomplicated lower segment cesarean section in her first pregnancy. She is allowed a trial of vaginal delivery. What is the earliest warning sign of scar rupture?

- a) Maternal dehydration
- b) Fresh vaginal bleeding
- c) Fetal distress
- d) Maternal tachycardia
- e) Severe continuous abdominal pain

The following are possible indications for cesarean hysterectomy EXCEPT:

- a) Placenta accreta
- b) Severe chorioamnionitis
- c) Severe uncontrollable postpartum hemorrhage during CS
- d) Rupture uterus by multiple tears
- e) Severe abdominal adhesions during CS

Application of forceps is appropriate in which of the following situations?

- a) Transverse lie, +3 station, cervix completely dilated, membranes ruptured
- b) Vertex at +3 station, cervix 8 cm dilated, membranes ruptured
- c) Breech at +3 station, cervix completely dilated, membranes intact
- d) Mentum anterior, +3 station, cervix completely dilated, membranes ruptured
- e) Vertex at +1 station, cervix completely dilated, membranes intact

The uterus becomes a pelvic organ after delivery by:

- a) Four weeks
- b) Three weeks
- c) Two weeks
- d) One week

Human Chorionic Gonadotropin (HCG) is produced by:

- a) Allantois
- b) Fetal yolk sac
- c) Amniotic membranes
- d) Trophoblastic cells
- e) Hypothalamus

Identify the placental abnormality shown in the figure:

- a) Velamentous insertion of the cord
- b) Succenturiate placenta
- c) Placenta circumvoluta
- d) Placenta membranacea
- e) Bilobed placenta

Your patient has microcytic anemia with a hemoglobin of 9 gm/dl and normal iron stores. What is the most likely diagnosis?

- a) Folate deficiency
- b) Vitamin B6 deficiency
- c) Vitamin B12 deficiency
- d) Thalassemia
- e) Iron deficiency anemia

A primigravida woman with mitral stenosis with no pulmonary hypertension is in labor at 39 weeks. She has dyspnea only in severe exertion. Fetus is in cephalic presentation. Cervix dilatation is 5 cm. She has two uterine contractions in 10 minutes. Which of the following should be avoided in this patient?

- a) Place the patient in semi sitting position
- b) Active management of third stage of labor
- c) Use of ergometrine in the third stage of labor
- d) Ventouse delivery in the second stage of labor if delivery does not occur in half hour
- e) Use of epidural analgesia for pain relief

SUPPORT 41

A 27-year-old G2P1 at 29 weeks gestational age who is being followed for Rh isoimmunization presents for her OB visit. An ultrasound reveals fetal ascites and a pericardial effusion. Which of the following can be another finding in fetal hydrops?

- a) Omphalocele
- b) Subcutaneous edema
- c) Hydronephrosis
- d) Hydrocephalus
- e) Oligohydramnios

A 20-year-old gravida 1 at 32 weeks presents for her routine OB visit. She has no medical problems. She is noted to have a blood pressure of 150/96, and her urine dip shows 1+ protein. She complains of a constant headache and vision changes that are not relieved with rest or a pain reliever. At the hospital her blood pressure is 158/98 and she is noted to have tonic-clonic seizure. Which of the following is indicated in the management of this patient?

- a) Cesarean delivery
- b) Antihypertensive therapy
- c) Phenytoin
- d) Magnesium sulfate
- e) Low-dose aspirin

A 28-year-old female with history of 8 weeks amenorrhea complains of vaginal bleeding and lower abdominal pain. On ultrasound examination there is gestational sac with absent fetal parts. The diagnosis is:

- a) Threatened abortion
- b) Placenta previa
- c) Ectopic pregnancy
- d) Blighted ovum
- e) Corpus luteum cyst

SUPPORT 41

A 32-year-old woman presents to the emergency department with abdominal pain and vaginal bleeding. Her last menstrual period was 8 weeks ago and her pregnancy test is positive. On examination she is tachycardic and hypotensive and her abdominal examination findings reveal peritoneal signs. A bedside abdominal ultrasound shows free fluid within the abdominal cavity. The decision is made to take the patient to the operating room for emergency exploratory laparotomy. Which of the following is the most likely diagnosis?

- a) Incomplete abortion
- b) Complete hydatidiform mole
- c) Ruptured ectopic pregnancy
- d) Missed abortion
- e) Incomplete hydatidiform mole

Primigravida with full term complains of fainting on lying down and she feels well when she turns to one side. This is due to:

- a) Increased intracranial pressure
- b) After heavy lunch
- c) Increased abdominal pressure
- d) Increased renal blood flow
- e) Inferior vena cava compression

During a routine return OB visit, an 18-year-old G1P0 patient at 23 weeks gestational age undergoes a urinalysis. The dipstick done by the nurse indicates the presence of trace glucosuria. All other parameters of the urine test are normal. Which of the following is the most likely etiology of the increased sugar detected in the urine?

- a) The patient has kidney disease
- b) The patient's urine sample is contaminated
- c) The patient's urinalysis is consistent with normal pregnancy
- d) The patient has a urine infection
- e) The patient has diabetes

SUPPORT 41

All of the following are causes of polyhydramnios EXCEPT:

- a) open spina bifida
- b) esophageal atresia
- c) Anencephaly
- d) diabetes mellitus
- e) post term pregnancy

A hypertensive pregnant woman at 34 weeks comes with history of abdominal pain, vaginal bleeding and loss of fetal movement. On examination the uterus is contracted with increased uterine tone. Fetal heart rate is absent. The most likely diagnosis is:

- a) Vasa previa
- b) Placenta previa
- c) Premature rupture of membranes
- d) Degeneration of uterine fibroid
- e) Abruptio placenta

Which of the following situations is the cause of this type of fetal heart deceleration shown in this CTG strip?

- a) Fetal head compression during labor
- b) Utero-placental insufficiency
- c) Fetal distress
- d) Fetal metabolic acidosis
- e) Umbilical cord compression



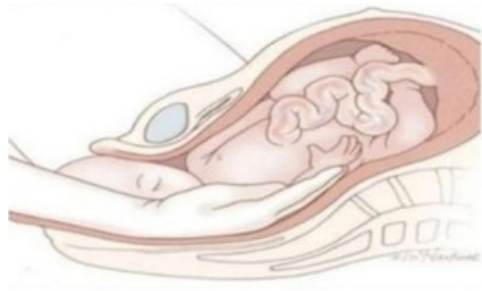
All of the following are components of biophysical profile EXCEPT:

- a) Fetal respiratory movement
- b) Oxytocin challenge test
- c) Amniotic fluid volume
- d) Fetal body movement
- e) Non stress test

SUPPORT 41

This image shows one of the maneuvers used in shoulder dystocia. This maneuver is:

- a) Screw maneuver
- b) Internal rotation
- c) Zavanelli maneuver
- d) Mc Roberts maneuver
- e) Delivery of posterior arm



Best time for diagnosis of chorionicity in twins pregnancy by ultrasonography is:

- a) 32-36 weeks
- b) 8-10 weeks
- c) 11-14 weeks
- d) 18-24 weeks
- e) 14-18 weeks

All of the following are contraindications for tocolysis EXCEPT:

- a) Anencephaly
- b) Chorioamnionitis
- c) Significant intrauterine fetal growth restriction
- d) Non-reassuring fetal heart rate
- e) Active uterine contractions at 33 weeks

A woman with dichorionic diamniotic twins attends antenatal clinic at 36 weeks. The first twin is in breech presentation and the second is in cephalic presentation. She has no other pregnancy complications. What is the most appropriate management?

- a) Perform external cephalic version of the first twin
- b) Await spontaneous onset of labor
- c) Perform elective cesarean section at 37 weeks
- d) Perform internal podalic version
- e) Induce labor at 37 weeks

SUPPORT 41

A 35-year-old woman, G2 P1+0 presents to antenatal clinic at 35 weeks of pregnancy with sudden gush of watery vaginal discharge. Sample of pooled liquid turned red litmus paper blue and the ferning was present. The temperature of the patient was 38°C and her pulse was 102 bpm.

What is the next step in management?

- a) Administer tocolytics
- b) Administer betamethasone
- c) Administer antibiotics
- d) Administer magnesium sulphate
- e) Place cervical cerclage

A woman is admitted at 36 weeks gestation following sudden episode of antepartum haemorrhage. The bleeding has ceased on admission. She is not pale. BP is 120/80 and pulse is 76 bpm. The uterus is soft and not tender. There are no palpable uterine contractions. FHR is 140/minute.

What is next step of management?

- a) Administer intravenous dexamethasone
- b) Examine the cervix using Dusco's speculum
- c) Perform cardiotocograph
- d) Pelvic examination under general anesthesia
- e) Ultrasound examination of the uterus

Which is NOT included in the management of the third stage of labor?

- a) Immediate cutting and cord clamping
- b) Uterotonic within one minute of delivery
- c) Massage of the uterus
- d) Controlled and sustained cord traction
- e) Direct oxytocin injection after delivery of the shoulders

SUPPORT 41

Mrs H, Gravida 2 para 1+0, presented to hospital in labor pains, on examination she had 3 uterine contractions of 20 seconds in 10 minutes, cervical dilatation 8 cm and FHR 145 per minute. What is the stage of labor?

- a) Stage II
- b) Stage III
- c) Stage I
- d) Stage IV
- e) Stage V

Cesarean section is mandatory in the following presentation:

- a) Face
- b) Vertex
- c) Occipito-posterior
- d) Breech
- e) Brow

A 32-year-old G2P1 presents to triage at 38 weeks and 5 days with ruptured membranes and meconium. She is having no contractions. The patient's first baby was delivered by Cesarean at term for footling breech presentation. She had discussed the possibility of a trial of labor after Cesarean section (TOLAC) during her prenatal visits and desires to proceed with this. What is this patient's greatest risk if she undergoes induction?

- a) Pulmonary embolism
- b) Uterine rupture
- c) Placental abruption
- d) Cord prolapse
- e) Meconium aspiration

SUPPORT 41

What is the name of this maneuver used for management of uterine atony?

- a) B Lynch compression suture
- b) Brandt Andrew
- c) Mauriceau Smellie Veit
- d) Credé
- e) Bimanual compression



A 20-year-old G1 at 38 weeks gestation presents with regular painful contractions every 3 to 4 minutes lasting 60 seconds. On pelvic examination, she is 3 cm dilated and 90% effaced; an amniotomy is performed and clear fluid is noted. The patient receives epidural analgesia for pain management. The fetal heart rate tracing is reactive. One hour later on repeat examination, her cervix is 5 cm dilated and 100% effaced. Which of the following is the best next step in her management?

- a) Begin pushing
- b) Initiate Pitocin augmentation for protracted labor
- c) Stop epidural infusion to enhance contractions and cervical change
- d) Perform cesarean delivery for inadequate cervical effacement
- e) No intervention; labor is progressing normally

In which presentation the fetal head is deflexed?

- a) Occipito-anterior position
- b) Brow presentation
- c) Compound presentation
- d) Occipito-posterior position
- e) Face presentation

What is the measurement of sub-occipito-bregmatic diameter?

- a) 10.5 cm
- b) 10 cm
- c) 9.5 cm
- d) 9 cm
- e) 11 cm

SUPPORT 41

A 25-year-old G1 at 37 weeks presents to labor and delivery with gross rupture of membranes. The fluid is noted to be clear and the patient is noted to have regular painful contractions every 2 to 3 minutes lasting for 60 seconds each. The fetal heart rate tracing is reactive. On cervical examination she is noted to be 4 cm dilated, 90% effaced with the presenting part at -3 station. The presenting part is soft and felt to be the fetal buttock. A quick bedside ultrasound reveals a breech presentation with both hips flexed and knees extended. What type of breech presentation is described?

- a) Frank
- b) Double footling
- c) Complete
- d) Knee presentation

All of the following are true about pathological retraction ring EXCEPT:

- a) Can be felt
- b) May be associated with intrauterine fetal death
- c) Can be relieved by anesthesia
- d) Associated with maternal dehydration
- e) Occurs between upper and lower uterine segment

All of the following are true for episiotomy EXCEPT:

- a) Allows widening of the birth canal
- b) Can be either midline or medio lateral
- c) Midline episiotomy bleeds less, easier to repair, and heals more quickly
- d) Involvement of the anal sphincter is classified as 4th degree perineal tear

SUPPORT 41

Regarding indications of cesarean hysterectomy, which of the following statements is NOT true?

- a) Operable carcinoma of the cervix during pregnancy
- b) Multiple uterine fibroids in relatively old pregnant woman who had completed her family
- c) Uncontrolled postpartum hemorrhage
- d) The mere presence of couvelaire uterus
- e) Placenta accreta encountered during cesarean section

All of the following are true regarding Caput succedaneum EXCEPT:

- a) Ill-defined edges
- b) Diffuse edema over more than one bone
- c) Dark overlying skin
- d) Disappears after two weeks
- e) Develops at birth

The nerve roots affected in this type of birth injury, are:

- a) C7, C8, T2
- b) C5, C6
- c) C1, C2, T1
- d) C7, C8, T1
- e) C8, T1



Name of the following instrument is:

- a) Piper's forceps
- b) Sim's speculum
- c) Doyen's retractor
- d) Metal catheter
- e) Keiland forceps



SUPPORT 41

G2 para 1+0 who had uncomplicated lower segment cesarean section in her first pregnancy in labor. She is allowed a trial of vaginal delivery. What is the earliest warning sign of scar rupture?

- a) Fresh vaginal bleeding
- b) Severe continuous abdominal pain
- c) Maternal tachycardia
- d) Fetal distress

Lochia in correct order during puerperium is:

- a) Serosa, rubra, alba
- b) Rubra, serosa, alba
- c) Alba, mucosa, serosa
- d) Alba, serosa, rubra

Prerequisites of outlet forceps include all the following EXCEPT:

- a) Uterus contracting
- b) Engaged head
- c) Station 0 or +1
- d) Fully dilated cervix

Forceps delivery can be done in all EXCEPT:

- a) Aftercoming head in breech delivery
- b) Deep transverse arrest
- c) Maternal heart disease
- d) Persistent mento posterior

Ventouse in second stage of labor is contraindicated in:

- a) Uterine inertia
- b) Preterm labor
- c) Heart disease
- d) Persistent occipito posterior position

SUPPORT 41

A 25 years old female, Gravida 5, para 0+3, with history of recurrent early pregnancy loss. Which of the following investigations is NOT ordered?

- a) creatinine clearance test
- b) Genetic testing and karyotyping
- c) testing for antiphospholipid syndrome
- d) testing for TORCH
- e) thyroid function test

A patient presents at 8 week's pregnant or at 8 week's gestation for her prenatal visit. Her main complaint is nausea and occasional vomiting. Her weight is the same as her prepregnancy weight. What do you tell her?

- a) She could have appendicitis and needs a computed tomography CT scan
- b) She has hyperemesis gravidarum
- c) she has increased size of the uterus that compresses sigmoid colon
- d) She has anorexia due to her lack of weight gain
- e) She has delayed gastric emptying due to progesterone effect

A 20-year-old G1 at 36 weeks is being monitored for preeclampsia; she rings the bell for the nurse because she is developing a headache. As you and the nurse enter the room, you witness the patient undergoing a tonic-clonic seizure. You secure the patient's airway, and within a few minutes the seizure is over. The patient's blood pressure monitor indicates a pressure of 160/110 mm Hg. Which of the following medications is recommended for the prevention of a recurrent eclamptic seizure?

- a) Nifedipine
- b) Pitocin
- c) Labetalol
- d) Magnesium sulfate
- e) Hydralazine

SUPPORT 41

A 22-year-old G1 at 32-week gestational age (GA) comes to see you for a routine prenatal visit. On examination, the patient appears healthy, fundal height is consistent with GA, and fetal heart tones (FHTs) are in the 140s. She is concerned because she received lab work from an insurance physician that shows decreased blood urea nitrogen, decreased creatinine, and trace glucose in her urine. You reassure the patient that these changes are normal due to:

- a) Mild gestational diabetes that can be managed by dietary changes
- b) The mass effect of the uterus on the bladder leading to compression and temporary physiologic changes
- c) the increased plasma volume exceeds the increased RBCs
- d) The direct effects of progesterone on the bladder during pregnancy
- e) An increase of 50% in the glomerular filtration rate that occurs in the first trimester and is maintained until delivery

Your patient has microcytic anemia with a hemoglobin of 9 and normal iron stores. What is the most likely diagnosis?

- a) folate deficiency
- b) thalassemia
- c) vitamin B6 deficiency
- d) vitamin B12 deficiency
- e) iron deficiency anemia

A 31-year-old G2P0010 at 5 week's gestation presents for her first prenatal visit complaining of vague abdominal cramping and bleeding that is slightly less than her normal menstrual period. The most appropriate steps you take to evaluate this patient are:

- a) ultrasound (U/S) and CBC
- b) Serial quantitative B-hCG
- c) Quantitative B-hCG, then U/S
- d) B. hCG and progesterone
- e) Qualitative beta human chorionic gonadotropin (hCG) and complete blood count (CBC)

SUPPORT 41

In case of diabetes mellitus with pregnancy:

- a) Insulin requirement decreases as pregnancy advances
- b) Oral hypoglycemics are preferred to insulin therapy
- c) The best screening test is urine glucose
- d) There is increased incidence of hydramnios and preeclampsia
- e) Congenital malformations are unrelated to diabetic control

Treatment of theca lutein ovarian cysts that are found in association with hydatiform mole is:

- a) Suction evacuation
- b) Ovariectomy
- c) Methotrexate
- d) Ovariotomy
- e) Ovarian cystectomy

Which of the following Leopold's maneuvers is shown in the image?

- a) Fundal grip
- b) Second pelvic grip
- c) Umbilical grip
- d) First pelvic grip



A primigravida woman with mitral stenosis with no pulmonary hypertension is in labor at 39 weeks. She has dyspnea only in severe exertion. Fetus is in cephalic presentation. Cervix dilatation is 5 cm. She has two uterine contractions in 10 minutes. Which of the following should be AVOIDED in this patient?

- a) Ventouse delivery in the second stage of labor if delivery does not occur in half hour
- b) Place the patient in semi sitting position
- c) Use of epidural analgesia for pain relief
- d) Use of ergometrine in the third stage of labor
- e) Active management of third stage of labor

SUPPORT 41

A 31-year-old G3P0202 presents at 28 weeks with possible rupture of membranes more than 2 days ago. She denies contractions and has had no bleeding. She says the baby has had decreased movements. The patient's vital signs are as follows: temperature (T) 38.5°C, heart rate (HR) 103 beats/min, blood pressure (BP) 122/76 mmHg, and O₂ saturation 98% on room air. FHR 180/min. The fundal height is 26 cm and the uterus is moderately tender. Sterile speculum examination reveals minimal pooling but positive ferning. What is the most likely diagnosis?

- a) Pelvic inflammatory disease
- b) Chorioamnionitis
- c) Preterm labor
- d) Placental abruption
- e) Placental insufficiency

A woman complains of sudden onset of watery vaginal discharge at a period of amenorrhea 32 weeks. The fetus is in cephalic presentation with no uterine contractions. FHR 140 beats per minute. What is the first step in management?

- a) Commence intravenous corticosteroids
- b) Commence intravenous tocolysis
- c) Commence broad spectrum antibiotics
- d) Perform ultrasound scan to assess amniotic fluid volume
- e) Perform sterile speculum examination

A hypertensive pregnant woman at 34 weeks comes with history of pain in abdomen, vaginal bleeding and loss of fetal movement. On examination the uterus is contracted with increased uterine tone. Fetal heart sound is absent. The most likely diagnosis is:

- a) Vasa previa
- b) Premature rupture of membranes
- c) Degeneration of uterine fibroid
- d) Abruptio placenta
- e) Placenta previa

SUPPORT 41

All of the following are causes of polyhydramnios EXCEPT:

- a) Open spina bifida
- b) Esophageal atresia
- c) Anencephaly
- d) Diabetes mellitus
- e) Post term pregnancy

A 26-year-old G1 with no prenatal care presents to labor and delivery because of bright red, painless vaginal bleeding that occurred after intercourse. Her gestational age of 29 weeks by last period is confirmed by fundal height and ultrasound that also reveals a complete placenta previa. There are no contractions and fetal heart tones (FHTs) are reactive. The bleeding has subsided. Hemoglobin is 12.6 g/dL and platelets are 199,000 cells/cc. Management of this patient includes which of the following?

- a) Immediate Cesarean section
- b) Immediate Pitocin induction of labor
- c) Vaginal examination
- d) Blood transfusion
- e) Hospitalization

Which of the following drugs is NOT used for tocolysis:

- a) Phenobarbitone
- b) Indomethacin
- c) Nifedipine
- d) Magnesium sulphate
- e) Ritodrine

SUPPORT 41

This image shows one of the maneuvers used in shoulder dystocia. This maneuver is:

- a) Delivery of posterior arm
- b) Internal rotation
- c) Screw maneuver
- d) Zavanelli maneuver
- e) McRoberts maneuver



Twins lambda sign is seen in:

- a) Diamniotic Dichorionic
- b) Dichorionic Monoamniotic
- c) None of the above
- d) Conjoined twins
- e) Monochorionic Diamniotic

Identify the given procedure done in labor room:



- a) Partogram
- b) Non stress test
- c) Contraction stress test
- d) Amnioinfusion
- e) Amniotomy

SUPPORT 41

Mrs H, Gravida 2 para 1+0, presented to hospital in labor pains, on examination she had 3 uterine contractions of 20 seconds in 10 minutes, cervical dilatation 10 cm with bearing down and FHR 145 per minute.

What is the stage of labor?

- a) Stage II
- b) Stage III
- c) Stage V
- d) Stage IV
- e) Stage I

The smallest diameter of true pelvis is:

- a) True conjugate diameter
- b) Intertuberous diameter
- c) Diagonal conjugate diameter
- d) Interspinous diameter

All of the following are true about pathological retraction ring EXCEPT:

- a) Can be relieved by anesthesia
- b) Associated with maternal dehydration
- c) Occurs between upper and lower uterine segment
- d) May be associated with intrauterine fetal death
- e) Can be felt

True about frank breech:

- a) Both are flexed
- b) Thigh flexed, knee extended
- c) Thigh extended, knee extended
- d) Buddha attitude
- e) More common in multipara

SUPPORT 41

A 30 years old female G2 para 1+0 at 38 weeks gestation presents with labor pains. On examination, cervix is 5 cm dilated and the fetus was found to be in face presentation with left mento-anterior position. The next step of management is:

- a) Jaw flexion-shoulder traction
- b) Expectant management
- c) Ventouse delivery
- d) Forceps
- e) Cesarean section

A female bleeds profusely soon after vaginal delivery. Placenta and membranes were expelled. IV fluids were initiated. Uterus is relaxed and fails to contract adequately after treatment with ecobolics. Her heart rate is 120 per minute. Blood pressure is 90/60. What is the next appropriate step of management?

- a) Ligate uterine arteries
- b) Do hysterectomy
- c) Carry out balloon tamponade
- d) Ligate internal iliac artery
- e) Apply B-Lynch suture

SUPPORT 41

The following techniques are used for delivery of the aftercoming head in breech presentation EXCEPT:

- a) Mauriceau Smellie Veit technique
- b) Jaw flexion shoulder traction
- c) Lovset maneuver
- d) Forceps delivery
- e) Burns Marshall

All of the following are true regarding cephalohematoma EXCEPT:

- a) Develops hours after birth
- b) Disappears within two days
- c) Normal overlying skin
- d) Well defined edges
- e) Localized over one bone

The following are indications of ergometrine EXCEPT:

- a) Incomplete abortion
- b) Subinvoluted uterus
- c) Uterine atony
- d) Inevitable abortion
- e) Cervical ripening

Advantages of ventouse over forceps include all of the following EXCEPT:

- a) Can be used before full cervical dilatation
- b) More compression force
- c) Preferred for cardiac patients
- d) Can be used for non-engaged head
- e) Less genital tract injuries

SUPPORT 41

The uterus becomes pelvic organ after delivery by:

- a) One week
- b) Three weeks
- c) Four weeks
- d) Two weeks

A 31 years old patient gravida 3 para 1+1, previous one spontaneous vaginal delivery, pregnant 42 weeks, came to the delivery room for assessment for eligibility for induction of labor. On vaginal examination, cervix was anterior, soft in consistency, closed, fully effaced (cervical length = 0) with fetal head station at +1. What is her Bishop score?

- a) 5
- b) 4
- c) 6
- d) 8
- e) 10

The following are possible indications for cesarean hysterectomy EXCEPT:

- a) Severe uncontrollable postpartum hemorrhage during CS
- b) Severe abdominal adhesions during CS
- c) Placenta accreta
- d) Rupture uterus by multiple tears
- e) Severe chorioamnionitis

G2 para 1+0 who had uncomplicated lower segment cesarean section in her first pregnancy in labor. She is allowed a trial of vaginal delivery. What is the earliest warning sign of scar rupture?

- a) Fetal distress
- b) Maternal tachycardia
- c) Maternal dehydration
- d) Fresh vaginal bleeding
- e) Severe continuous abdominal pain

SUPPORT 41

Application of forceps is appropriate in which of the following situations?

- a) Vertex at +1 station, cervix completely dilated, membranes intact
- b) Mentum anterior, +3 station, cervix completely dilated, membranes ruptured
- c) Breech at +3 station, cervix completely dilated, membranes intact
- d) Vertex at +3 station, cervix 8 cm dilated, membranes ruptured
- e) Transverse lie, +3 station, cervix completely dilated, membranes ruptured

The following are true about this instrument EXCEPT:

- a) Obtain sample for endometrial biopsy
- b) Assess the length of uterine cavity
- c) Confirm if the uterus is anteverted or retroverted
- d) May cause uterine perforation if not used gently
- e) Used preliminary to uterine curettage

A patient of 36 weeks gestation presents with hypertension, blurring of vision and headache. Her blood pressure reading was persistent 180/120. Proteinuria +3. How will you manage this patient:

- a) Admit the patients, start antihypertensive and continue pregnancy till term
- b) Admit the patient and observe
- c) Start antihypertensive and follow up in outpatient department
- d) Admit the patient, start antihypertensive, $Mgso_4$, and terminate the pregnancy

SUPPORT 41

With regards acute pyelonephritis with pregnancy, all are true EXCEPT:

- a) More common in second trimester of pregnancy
- b) Most common isolates E. coli
- c) Responds to cephalosporins
- d) The incidence of left kidney involvement is more than the right kidney

A 28 years old female with history of 8 weeks amenorrhea complains of vaginal bleeding and lower abdominal pain. On ultrasound examination there is gestational sac with absent fetal parts. The diagnosis is:

- a) Threatened abortion
- b) Placenta previa
- c) Ectopic pregnancy
- d) Blighted ovum
- e) Corpus luteum cyst

Primigravida with full term complains of fainting on lying down and she feels well when she turns to one side. This is due to:

- a) Increased intracranial pressure
- b) Inferior vena cava compression
- c) Increased abdominal pressure
- d) After heavy lunch

All the following are sure signs of pregnancy EXCEPT:

- a) Auscultation of FIHS
- b) Umbilical soufflé
- c) Palpation of fetal parts
- d) Uterine soufflé

SUPPORT 41

If the head is allowed to extend before this sign appears, the head will be delivered by:

- a) Mentoverical diameter
- b) Suboccipito bregmatic diameter
- c) Submentobregmatic diameter
- d) Suboccipitofrontal diameter
- e) Occipitofrontal diameter

A 30 years old female G2 para 1+0 at 38 weeks gestation presents with labor pains. On examination, cervix is 5 cm dilated and the fetus was found to be in face presentation with left mento-anterior position. The next step of management is:

- a) Cesarean section
- b) Ventouse delivery
- c) Forceps
- d) Expectant management
- e) Jaw flexion- shoulder traction

The smallest diameter of true pelvis is:

- a) True conjugate diameter
- b) Intertuberous diameter
- c) Diagonal conjugate diameter
- d) Interspinous diameter

SUPPORT 41

A 32-year-old G2P 1 presents to triage at 38 weeks and 5 days with ruptured membranes and meconium. She is having no contractions. The patient's first baby was delivered by Cesarean at term for footling breech presentation. She had discussed the possibility of a trial of labor after Cesarean section (TOLAC) during her prenatal visits and desires to proceed with this. What is this patient's greatest risk if she undergoes induction?

- a) Cord prolapse
- b) Placental abruption
- c) Uterine rupture
- d) Pulmonary embolism
- e) Meconium aspiration

A female bleeds profusely soon after vaginal delivery. Placenta and membranes were expelled. IV fluids were initiated. Uterus is relaxed and fails to contract adequately after treatment with ergonovine. Her HR = 120 per minute. Blood pressure = 90/60. What is the next appropriate step of management?

- a) Carry out balloon tamponade
- b) Ligate uterine arteries
- c) Ligate internal iliac artery
- d) Apply B-Lynch suture
- e) Do hysterectomy

The following techniques are used for delivery of the Aftercoming head in breech presentation EXCEPT:

- a) Jaw flexion shoulder traction
- b) Mauriceau Smillie Veit technique
- c) Lovset maneuver
- d) Burns Marshall's technique
- e) Forceps delivery

SUPPORT 41

A 24-year-old G1 at term is taken to the operating room for a stat Cesarean for fetal distress with fetal heart tones (FHTs) in the 60s for over 10 minutes despite repositioning and other efforts. The newborn has an HR of less than 100, a slow irregular respiratory rate, flaccid muscle tone, a weak cry, and blue coloring of the body and limbs. What is the appropriate Apgar score?

- a) 3
- b) 1
- c) 4
- d) 5
- e) 2

All of the following are true about pathological retraction ring EXCEPT:

- a) Associated with maternal dehydration
- b) Can be relieved by anesthesia
- c) Can be felt
- d) Occurs between upper and lower uterine segment
- e) May be associated with intrauterine fetal death

Mrs H, Gravida 2 para 1+0, presented to hospital in labor pains, on examination she had 3 uterine contractions of 20 seconds in 10 minutes, cervical dilatation 10 cm with bearing down and FHR 145 per minute.

What is the stage of labor?

- a) Stage I
- b) Stage III
- c) Stage V
- d) Stage IV
- e) Stage II

True about frank breech:

- a) More common in multipara
- b) Thigh extended, knee extended
- c) Both are flexed
- d) Buddha attitude
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SUPPORT 41

The following are true about this instrument EXCEPT:

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- b) Assess the length of uterine cavity
- c) Used preliminary to uterine curettage
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- e) Obtain sample for endometrial biopsy

Application of forceps is appropriate in which of the following situations?

- a) Breech at +3 station, cervix completely dilated, membranes intact
- b) Mentum anterior, +3 station, cervix completely dilated, membranes ruptured
- c) Transverse lie, +3 station, cervix completely dilated, membranes ruptured
- d) Vertex at +3 station, cervix +8 cm dilated, membranes ruptured
- e) Vertex at +1 station, cervix completely dilated, membranes intact

The nerve roots affected in Klumpke's palsy are:

- a) C1, C2, T1
- b) C8, T1
- c) C7, C8, T1
- d) C5, C6
- e) C7, C8, T2

The following are indications of ergometrine EXCEPT:

- a) Cervical ripening
- b) Subinvoluted uterus
- c) Uterine atony
- d) Inevitable abortion
- e) Incomplete abortion

SUPPORT 41

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SUPPORT 41

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- a) 6
- b) 10
- c) 8
- d) 4
- e) 5

The uterus becomes pelvic organ after delivery by:

- a) Three weeks
- b) Two weeks
- c) One week
- d) Four weeks

All of the following are sequelae of macrosomic baby EXCEPT:

- a) Meconium aspiration syndrome
- b) Erb's palsy
- c) Shoulder dystocia
- d) Obstructed labor
- e) Disseminated intravascular coagulopathy

All of the following are ultrasound features of intrauterine fetal demise EXCEPT:

- a) Absent fetal movement
- b) Collapse of the thorax
- c) Gases in the intestine
- d) Turtle sign
- e) Spalding sign

SUPPORT 41

Monozygotic twins have the following characteristics EXCEPT:

- a) More susceptible to twin to twin transfusion syndrome
- b) Accounts for 20% of twin pregnancies
- c) Have the same gender
- d) Have different fingerprints
- e) Have different blood groups

All of the following maneuvers are performed in shoulder dystocia EXCEPT:

- a) Fundal pressure
- b) Episiotomy
- c) Rotation of anterior shoulder
- d) Delivery of posterior arm
- e) Mc-Robert's position

Which of the following diseases can cause polyhydramnios?

- a) Duodenal atresia
- b) IUGR
- c) Post maturity
- d) Placental insufficiency
- e) Absent kidneys

With reference to fetal heart rate, a non-stress test is considered reactive when:

- a) Three heart rate accelerations are noted in 30 minutes
- b) One fetal heart rate acceleration is noted in 20 minutes
- c) Two fetal heart rate accelerations are noted in 20 minutes
- d) Two fetal heart rate accelerations are noted in 10 minutes
- e) Fetal heart accelerations during uterine contractions

SUPPORT 41

The following vaccines are safe during pregnancy EXCEPT:

- a) Influenza
- b) Hepatitis B
- c) Pneumococcus
- d) Tetanus
- e) Rubella

A primigravida complains of lower abdominal pain at a period of amenorrhea of 32 weeks. The fetus is in cephalic presentation. She has 1–2 uterine contractions per 10 minutes. Cervix is 2 cm dilated and 75% effaced with intact membranes. What is the preferred next step of management?

- a) Allow labor to progress normally
- b) Start intravenous broad spectrum antibiotics
- c) Commence intramuscular corticosteroids and oral nifedipine
- d) Commence intravenous ecobolics
- e) Start intramuscular corticosteroids and intravenous ritodrine

A woman is admitted to hospital with a history of dribbling for 24 hours at a period of amenorrhea of 32 weeks. Speculum examination confirms dribbling. The liquor is offensive. Her temperature is 38°C. High reactive protein. No uterine contractions. FHR is 170 per minute. What is the most appropriate treatment?

- a) Augment labor with intravenous infusion of oxytocin
- b) Administer broad spectrum antibiotics and corticosteroids and manage conservatively if the temperature decreases
- c) Repeat C-reactive protein 24 hours after commencing broad spectrum antibiotics
- d) Perform cesarean section immediately after administering 1 dose of corticosteroids and broad spectrum antibiotics

SUPPORT 41

The following abnormal findings can be found in IUGR baby EXCEPT:

- a) Increased maternal serum alpha fetoprotein
- b) Absent umbilical artery diastolic blood flow
- c) Oligohydramnios
- d) Change in Head/Abdomen ratio
- e) Absent uterine artery diastolic notch

SUPPORT 41

The placenta of twins can be:

- a) Dichorionic and monoamniotic in dizygotic twins
- b) Dichorionic and monoamniotic in monozygotic twins
- c) Monochorionic and monoamniotic in dizygotic twins
- d) Monochorionic and diamniotic in monozygotic twins

This image shows one of the maneuvers used in shoulder dystocia. This maneuver is:

- a) Screw maneuver
- b) Internal rotation
- c) Delivery of posterior arm
- d) Zavanelli maneuver



A 35-year-old woman, G2 P1+0, presents to antenatal clinic at 35 weeks of pregnancy with sudden gush of watery vaginal discharge. A sample of pooled liquid turned red litmus paper blue and ferning was present. The patient's temperature was 38°C and her pulse was 102 bpm. What is the next step in management?

- a) Administer antibiotics
- b) Place cervical cerclage
- c) Administer betamethasone
- d) Administer tocolytics

Primigravida at 37 weeks gestation reported to labor room with central placenta previa with heavy vaginal bleeding. The fetal heart rate was normal at the time of examination. The best management option for this patient is:

- a) Expectant management
- b) Induction and vaginal delivery
- c) Cesarean section
- d) Induction and forceps delivery

Old Quizzes

A woman complains of right-sided abdominal pain and mild vaginal bleeding for one day after a period of amenorrhea of 6 weeks. Her general condition is satisfactory. There is tenderness in the right iliac fossa. The urine pregnancy test is positive. Transvaginal ultrasound scan does not reveal any adnexal mass or intrauterine pregnancy. What is the next step of management?

- a) Repeat ultrasound scan after 7 days
- b) Start treatment with methotrexate
- c) Perform two serum beta HCG tests 48 hours apart
- d) Perform laparoscopic examination

Primigravida with full term complains of fainting on lying down and she feels well when she turns to one side. This is due to:

- a) Increased abdominal pressure
- b) Inferior vena cava compression
- c) Increased intracranial pressure
- d) After heavy lunch

A 30-year-old para 2+2 female has been diagnosed to have complete vesicular mole. The treatment of choice would be:

- a) Total hysterectomy
- b) Suction evacuation
- c) Radiotherapy
- d) Expectant management

All the following are sure signs of pregnancy EXCEPT:

- a) Palpation of fetal parts
- b) Auscultation of fetal heart sounds (FIHS)
- c) Umbilical souffle
- d) Uterine soufflé

SUPPORT 41

Human Chorionic Gonadotropin (HCG) is produced by:

- a) Amniotic membranes
- b) Fetal yolk sac
- c) Hypothalamus
- d) Trophoblastic cells

A 28-year-old female with history of 8 weeks amenorrhea complains of vaginal bleeding and lower abdominal pain. On ultrasound examination there is gestational sac with absent fetal parts. The diagnosis is:

- a) Corpus luteum cyst
- b) Ectopic pregnancy
- c) Threatened abortion
- d) Blighted ovum

With regards to acute pyelonephritis with pregnancy, all are true EXCEPT:

- a) More common in second trimester of pregnancy
- b) Most common isolate is E. coli
- c) Responds to cephalosporins
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A patient of 36 weeks gestation presents with hypertension, blurring of vision, and headache. Her blood pressure reading was persistent 180/120. Proteinuria +3. How will you manage this patient?

- a) Start antihypertensive and follow up in outpatient department
- b) Admit the patient, start antihypertensive and continue pregnancy till term
- c) Admit the patient and observe
- d) Admit the patient, start antihypertensive, MgSO_4 , and terminate the pregnancy

SUPPORT 41

In a pregnant woman with heart disease, all the following are to be done EXCEPT:

- a) IV methergin after delivery
- b) Prophylactic antibiotic
- c) IV frusemide postpartum
- d) Shortening second stage of labor

Uterine blood flow at term is:

- a) 750 ml/min
- b) 250 ml/min
- c) 350 ml/min
- d) 50 ml/min

A woman complains of right-sided lower abdominal pain and mild vaginal bleeding after a period of amenorrhea of 6 weeks. Her general condition is satisfactory. Serum HCG is 2200 IU/ml. Transvaginal ultrasound reveals adnexal mass 20 mm with no cardiac activity, and no intrauterine pregnancy. The most appropriate management is:

- a) Perform laparoscopic salpingostomy
- b) Treat with intramuscular methotrexate
- c) Treat with intramuscular methyl ergometrine
- d) Perform laparoscopic salpingectomy

A 25-year-old female, Gravida 5, para 0+3, with history of recurrent early pregnancy loss. Which of the following investigations is not ordered?

- a) Thyroid function test
- b) Testing for antiphospholipid syndrome
- c) Testing for TORCH
- d) Genetic testing and karyotyping

SUPPORT 41

Regarding the process of fertilization, which of the following statement is correct?

- a) Fertilization occurs most commonly in isthmus of the fallopian tube
- b) Implantation occurs most commonly on posterior wall near the fundus
- c) Implantation begins 14 days after fertilization
- d) Morula enters uterus on day 7

All of the following are maternal changes during pregnancy EXCEPT:

- a) Increased fibrinogen
- b) Increased cardiac output
- c) Increased glomerular filtration rate
- d) Increased alveolar residual volume

Which of the following Leopold's maneuvers is shown in the image?



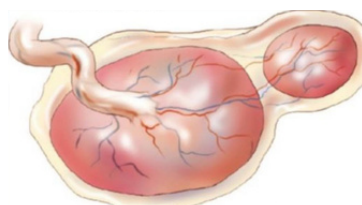
- a) Umbilical grip
- b) Second pelvic grip
- c) Fundal grip
- d) First pelvic grip

The following are related to uterine soufflé EXCEPT:

- a) It can be heard in a big fibroid
- b) It is due to increased blood flow through the umbilical artery
- c) The sound is synchronous with maternal pulse
- d) It is a soft blowing systolic murmur heard on the sides of the pregnant uterus

Identify the placental abnormality shown in the figure:

- a) Succenturiate placenta
- b) Placenta membranacea
- c) Bilobed placenta
- d) Velamentous insertion of the cord



SUPPORT 41

A 42-year-old female, P4+2, has been diagnosed with complete vesicular mole. The treatment of choice would be:

- a) Chemo radiotherapy
- b) Hysterectomy
- c) Chemotherapy
- d) Radiotherapy

A primigravida 32-year-old female, admitted to the antenatal ward at 35 weeks, her BP was 160/110. Her urine does not contain albumin. Which of the following drugs should not be used in her management?

- a) Methyldopa
- b) Furosemide
- c) Labetalol
- d) Hydralazine

The most common rheumatic heart disease associated with pregnancy is:

- a) Mitral regurgitation
- b) Mitral stenosis
- c) Aortic regurgitation
- d) Aortic stenosis

A hypertensive pregnant woman at 34 weeks comes with history of abdominal pain, vaginal bleeding and loss of fetal movement. On examination, the uterus is contracted with increased uterine tone. Fetal heart rate is absent. The most likely diagnosis is:

- a) Abruptio placenta
- b) Placenta previa
- c) Premature rupture of membranes
- d) Vasa previa

SUPPORT 41

All are causes of intrauterine growth restriction EXCEPT:

- a) Pregnancy induced hypertension
- b) Tubal ectopic pregnancy
- c) Maternal heart disease
- d) Anemia

A pregnant woman is found to have excessive accumulation of amniotic fluid. Such polyhydramnios is likely to be associated with all of the following conditions EXCEPT:

- a) Twins pregnancy
- b) Bilateral renal agenesis
- c) Microcephaly
- d) Oesophageal atresia

All of the following are contraindications to tocolysis EXCEPT:

- a) Anencephaly
- b) Placenta previa
- c) Fetal distress
- d) Chorioamnionitis

Complications of diabetes in pregnancy include all the following EXCEPT:

- a) Neural tube defect
- b) Intrauterine growth restriction (IUGR)
- c) Shoulder dystocia

Complications of diabetes in pregnancy include all the following EXCEPT:

- a) Intrauterine growth restriction (IUGR)
- b) Shoulder dystocia
- c) Hyperglycemia in newborn
- d) Macrosomia

SUPPORT 41

A Gravida 1, para 0+0 woman, attends antenatal clinic for the booking visit at 12 weeks. Her blood group is B Rhesus negative. What is the next step in management?

- a) Perform Rhesus antibody titre
- b) Determine the husband's blood group
- c) Perform ultrasound scan
- d) Determine fetal blood group

Techniques of delivery of aftercoming head in breech presentation include all EXCEPT:

- a) Piper's Forceps
- b) External cephalic version
- c) Burns Marshall's technique
- d) Jaw flexion shoulder traction

The smallest diameter of the true pelvis is:

- a) Diagonal conjugate diameter
- b) True conjugate diameter
- c) Intertuberous diameter
- d) Interspinous diameter

A woman undergoes induction of labor with amniotomy followed by oxytocin infusion at a period of amenorrhea 42 weeks. Six hours later she is getting 4 contractions per 10 minutes and the fetal heart rate drops to 100 per minute. What is the first step in management?

- a) Stop oxytocin infusion
- b) Perform cardiotocograph
- c) Change the position of the mother
- d) Administer oxygen to the mother

SUPPORT 41

In which fetal presentation can vaginal delivery be expected?

- a) Shoulder presentation
- b) Transverse lie
- c) Brow presentation
- d) Face presentation when the chin lies under the symphysis pubis

Cesarean section is mandatory in the following presentation:

- a) Face
- b) Cephalic
- c) Brow
- d) Vertex

Most common cause of postpartum hemorrhage is:

- a) Trauma
- b) Retained placental products
- c) Uterine atony
- d) Coagulopathy

In pathological retraction ring: All are true EXCEPT:

- a) Indicates obstructed labour and needs caesarean section
- b) Appears as a visible & palpable groove rising across the abdomen
- c) Can be felt vaginally
- d) More common in multiparous women

A female bleeds profusely soon after delivery. Placenta and membranes were expelled. IV fluids were initiated. Uterus is relaxed and fails to contract adequately after treatment with ecobolics. Her HR = 120 per minute, Blood pressure = 90/60. What is the next appropriate step of management?

- a) Ligate internal iliac artery
- b) Do hysterectomy
- c) Apply B-Lynch suture
- d) Carry out balloon tamponade

SUPPORT 41

Mrs H, Gravida 2 para 1+0, presented to hospital in labor pains, on examination she had 3 uterine contractions of 20 seconds in 10 minutes, cervical dilatation 6 cm and FHR 145 per minute. What is the stage of labor?

- a) Stage III
- b) Stage II
- c) Stage V
- d) Stage IV
- e) Stage I

Abnormal uterine action includes all of the following EXCEPT:

- a) Constriction ring
- b) Cervical insufficiency
- c) Colicky uterus
- d) Pathological Retraction ring
- e) Cervical dystocia

All of the following are true regarding cephalohematoma EXCEPT:

- a) Develops hours after birth
- b) Well-defined edges
- c) Normal overlying skin
- d) Disappears within two days
- e) Localized over one bone

G2 para 1+0 who had uncomplicated lower segment cesarean section in her first pregnancy in labor. She is allowed a trial of vaginal delivery. What is the earliest warning sign of scar rupture?

- a) Fetal distress
- b) Fresh vaginal bleeding
- c) Severe continuous abdominal pain
- d) Maternal tachycardia

SUPPORT 41

The uterus becomes pelvic organ after delivery by:

- a) Four weeks
- b) Two weeks
- c) Three weeks
- d) One week

Ventouse in second stage of labor is contraindicated in:

- a) Persistent occipito posterior position
- b) Uterine inertia
- c) Heart disease
- d) Preterm labor

Lochia in correct order during puerperium is:

- a) Alba, mucosa, serosa
- b) Alba, serosa, rubra
- c) Rubra, serosa, alba
- d) Serosa, rubra, alba

All of the following are true for episiotomy EXCEPT:

- a) Involvement of the anal sphincter is classified as 4th degree perineal tear
- b) Midline episiotomy bleeds less, easier to repair, and heals more quickly
- c) Can be either midline or medio-lateral
- d) Allows widening of the birth canal

The nerve roots affected in Klumpke's palsy are:

- a) C7, C8, T1
- b) C8, T1
- c) C1, C2, T1
- d) C7, C8, T2
- e) C5, C6

SUPPORT 41

Forceps delivery can be done in all EXCEPT:

- a) Maternal heart disease
- b) Aftercoming head in breech delivery
- c) Deep transverse arrest
- d) Persistent mento posterior

Regarding indications of cesarean hysterectomy, which of the following statements is NOT true?

- a) Placenta accreta encountered during cesarean section
- b) Multiple uterine fibroids in relatively old pregnant woman who had completed her family
- c) The mere presence of couvelaire uterus
- d) Uncontrolled postpartum hemorrhage
- e) Operable carcinoma of the cervix during pregnancy

Prerequisites of outlet forceps include all the following EXCEPT:

- a) Uterus contracting
- b) Engaged head
- c) Station 0 or +1
- d) Fully dilated cervix

وَأَخِرُ دَعْوَاهُمْ أَنِ الْحَمْدُ لِلَّهِ رَبِّ الْعَالَمِينَ